NORTHWEST LAS VEGAS
7455 W. Washington Ave Suite 160
Las Vegas, Nevada 89128

Phone: (702) 878-0393
www.nevadaorthopedic.com

HENDERSON
1505 Wigwam Pkwy Suite 330
Henderson, Nevada 89074

Physical/Occupational Therapy Order

Order Date: 04/08/2026

Patient Information

Suzanne M. Bell
161 W Longacres Dr
Henderson, NV 89015-7913
(702) 580-8108
Gender: Female Date of Birth: 10/01/1968

Diagnosis

Unspecified fracture of upper end of right humerus, subsequent encounter for fracture with routine healing (S42.201D)
Displaced oblique fracture of shaft of right ulna, subsequent encounter for closed fracture with routine healing (S52.231D)

Patient Insurance Information

MEDICARE (855) 609-9960
Group #NONE
Plan #2TE2JD9TV88

HPES NV MEDICAID (800) 942-6511
Group #NONE
Plan #62263100001

Physical/Occupational Therapy Facility

Orders

PT: Eval & Treat w/ Modalities Frequency/Duration: HOME HEALTH EVAL AND TREAT: SKILLED NURSING: HOME PT RT UPPER EXTREMITY ROM, Strengthening, 2XWK FOR 6-8WKS ADD STRETCHING AND GAIT TRAINING
on behalf of JASON A. ZARN, PA-C

OTHER/COMMENTS:

Fax Reports to:

Patient Information

Patient Name:	Suzanne M. Bell	Social Security #:	565-13-8862	Marital Status:	Undefined
Birth Date:	10/01/1968	Language:	English	Referred by:	ABRAHAM BAIDOO, PA-C
		Race:	White		
Gender:	Female	Ethnicity:	Not Hispanic or Latino	PCP:	ABRAHAM BAIDOO, MD
Home Address:	161 W Longacres Dr Henderson, NV 89015-7913 Phone: (702) 580-8108 Mobile: (702) 580-8108 Email: ral1roz@aol.com	Work Address:	(not available)	Employer: Occupation: Employment Status:	RETIRED None Unemployed
Guarantor Name:	Suzanne M. Bell	Relation to Guarantor:	Self	Guarantor Information:	161 W Longacres Dr Henderson, NV 89015-7913 Phone: (702) 580-8108 Email: ral1roz@aol.com

Insurance Information

Carrier Information	Plan Information	Subscriber Information
MEDICARE PO Box 6776 Fargo, ND 58108-6776 Phone: (855) 609-9960 Plan Type: Undefined	Group #:NONE Plan #:2TE2JD9TV88 Enrollement Date:01/01/2008 Group #:NONE Plan #:62263100001 Enrollement Date:10/01/2025	Name: Suzanne M. Bell DOB: 10/01/1968 Address: 161 W Longacres Dr Henderson, NV 89015-7913 Phone: (702) 580-8108
HPES NV MEDICAID PO Box 30042 Reno, NV 89520-3042 Phone: (800) 942-6511 Plan Type: Undefined	Group #:NONE Plan #:2TE2JD9TV88 Enrollement Date:01/01/2008 Group #:NONE Plan #:62263100001 Enrollement Date:10/01/2025	Name: Suzanne M. Bell DOB: 10/01/1968 Address: 161 W LONGACRES DR Henderson, NV 89015-7913 Phone: (702) 580-8108

Emergency Contacts

ROSALINE BITTER, Mother(760) 267-0738
Ral1roz@Aol.Com Ral1roz@Aol.Com, Sibling(760) 267-0738

Orthopedic Evaluation

History of Present Illness

The patient is a 57 year old female who presents for a follow-up visit.

Patient History

Allergies

- Azithromycin *CHEMICALS* [Drug allergy]
- Sulfa 10 *OPHTHALMIC AGENTS* [Drug allergy]
- sulfamethoxazole []: Nausea, Unconsciousness
- Erythromycin Base TABS []: Nausea, Unconsciousness
- Amprenavir/Sulfonamide []: Nausea, Unconsciousness

Medications

- topiramate (100mg tablet, oral) Active.
- TEGretoL (200mg tablet, oral) Active.
- sertraline (100mg tablet, oral) Active.
- upadacitinib (15mg Tablet, Extende, oral) Active.
- predniSONE (5mg tablet, oral) Active.
- meloxicam (7.5mg tablet, oral) Active.
- PHENobarbitaL (64.8mg tablet, oral) Active.
- Medications Reconciled.

Past Surgical

- Hip and/or Femur Surgery
- Metal Implant(s)
- Wrist Surgery

Social

- Alcohol Assessment: No Current Alcohol Problem 02/01/2024
- Alcohol use: None 02/01/2024
- Assistance Status: Home Assistance Available 02/01/2024
- Contract: No Active Pain Contract 02/01/2024
- Current Rehab Status: No Current Drug Rehab 02/01/2024
- Current work status: Not Currently Employed 02/01/2024
- Disability Status: Disabled 02/01/2024
- Drug Assessment: No Ilicit Drug Use 02/01/2024
- Drug Dependency: No Drug Dependence 02/01/2024
- Exercise: daily 02/01/2024
- Hand Dominance: Right Handed 02/01/2024
- Past Rehab Status: No Past Drug Rehab 02/01/2024
- Pregnancy Status: Not Pregnant 02/01/2024
- Surgical Blood Transfusion: Approves surgical blood transfusion 02/01/2024
- Tobacco use: Never smoker 02/01/2024

Family

- Heart Attack: Father
- Hypertension: Father
- Osteoarthritis: Mother

Other Past Medical

Epilepsy/Seizures
Fibromyalgia
Rheumatoid Arthritis
Site Specific Fracture Treatment: no
Bracing Phreesia 08/20/2025

Problem List

Displaced oblique fracture of shaft of right ulna, subsequent encounter for closed fracture with routine healing

Vitals

04/08/2026 01:32 PM
Weight: 75 lb Height: 59 in
Body Mass Index: 15.15 kg/m²
Pain Level: /10

Physical Exam

History of present illness: Very pleasant 57-year-old female with history significant for severe rheumatoid arthritis, osteoporosis and end-stage right shoulder arthritis presents to my office for reevaluation of her right shoulder and right ulnar shaft fracture. Patient notes the date of injury for the shoulder fracture was 6 weeks ago. Date of injury for the ulnar shaft fracture was approximately 5 to 6 months ago. Patient reports the shoulder feels much better as well as the ulnar shaft fractures.

General appearance: The patient is awake, alert and oriented ×4. The patient is in no apparent distress.

Physical examination: On examination of this patient's right shoulder finds no obvious gross deformity. There is expected resolved postinjury swelling and ecchymosis to both the shoulder and the forearm. 2+ RP. No wrist drop. No sensory deficits. Brisk cap refill. Stiff range of motion of the elbow and shoulder albeit improved from prior visit. Good motion of the forearm without pain.. Good motion of the wrist and phalanges. The patient is grossly neurovascularly intact. No pain to palpation to the forearm or the shoulder.

Radiographic examination: X-rays of this patient's right shoulder 2 views finds a mildly impacted and comminuted proximal humerus fracture with good congruency of the GH joint. End-stage arthritis is present of the GH joint. Interval bony healing seen. 2 views of the right forearm demonstrates healed ulnar shaft fracture with serial imaging.

Plan:
Very pleasant 57-year-old female has acute right shoulder proximal humerus fracture with comminution and mild impaction and right forearm ulnar shaft fracture, stable and improved. I recommend home health PT. Patient will follow-up in 6 weeks for recheck. DC sling. The ulna shaft fracture appears to be healed with repeat imaging. She may resume normal daily activities.

Patient does voice understanding and agrees with the above plan of care. All questions were answered to the patient's satisfaction.

The patient will follow up in clinic with myself in 6 weeks for recheck
The patient knows to call and/or return to clinic with any questions and/or concerns

The patient was seen today and examined by PA Zarn and discussed fully with Dr. Holman Chan, along with plan collaboration

Future plan:
Order 2 views right shoulder next visit

Thank you for allowing me to participate in the care of Mrs. Bell

The dictated information, contained herein, was transcribed with voice recognition software. Although reviewed, the content may contain errors and/or omissions.

Assessment and Plan

Unspecified fracture of upper end of right humerus, subsequent encounter for fracture with routine healing (S42.201D)
Displaced oblique fracture of shaft of right ulna, subsequent encounter for closed fracture with routine healing (S52.231D)

Orders
PT: Eval & Treat w/ Modalities
Shoulder X-Ray, 2 Views (Taken and Read) - RT
Forearm X-Ray, 2 Views (Taken and Read) - RT

Additional Instructions
Follow up in 6 weeks

Electronically signed by: JASON A. ZARN PA-C
on 04/08/2026

592490
04/12/24

NOT EDITED OR REVIEWED
BY PHYSICIAN
MUST OBTAIN OFFICIAL
RECORDS FROM HOSPITAL

FLAMINGO SURGERY CENTER
2565 E. Flamingo Road
Las Vegas, NV 89121
Tel: (702) 697-7900 Fax: (702) 796-6250

OPERATIVE REPORT

PATIENT NAME: BELL, SUZANNE
DATE OF BIRTH: 10/01/1968
DATE OF SURGERY: 04/12/2024
PATIENT MRN #: 351578
PHYSICIAN: KEVIN GOLSHANI, M.D.

PREOPERATIVE DIAGNOSES:

1. Left volar wrist mass.
2. Left dorsal wrist mass.

POSTOPERATIVE DIAGNOSES:

1. Left volar wrist mass.
2. Left index finger EDC tenosynovitis.
3. Left middle finger EDC tenosynovitis.
4. Left ring finger EDC tenosynovitis.
5. Left small finger EDC tenosynovitis.
6. Left ring finger EDC tendon attritional tear.
7. Left small finger EDC tendon attritional tear.

PROCEDURES PERFORMED:

1. Left index finger EDC tenosynovectomy.
2. Left middle finger EDC tenosynovectomy.
3. Left ring finger EDC tenosynovectomy.
4. Left small finger EDC tenosynovectomy.
5. Left ring finger EDC tendon repair.
6. Left small finger EDC tendon repair.
7. Left volar mass excision.

SURGEON: Kevin Golshani, M.D.

ANESTHESIA: General.

ANESTHESIOLOGIST: Scott Duong, M.D.

ASSISTANT: None.

URINE OUTPUT: None.

IV ANTIBIOTICS: Ancef 2 g.

IV FLUIDS: None.

BLOOD LOSS: Less than 5 mL.

COMPLICATIONS: None.

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PATIENT MRN #: 351578
PHYSICIAN: KEVIN GOLSHANI, M.D.

SPECIMEN: None.

FINDINGS: Left volar wrist mass, left index, middle, ring, and small finger inflamed tenosynovitis with rice type bodies encompassing all tendons; inflamed dorsal wrist capsule globally, and attritional rupture of the ring and small extensor tendon.

DISPOSITION: Stable to PACU.

DESCRIPTION OF PROCEDURE: The patient was identified in the preoperative holding area where the correct patient, laterality, and procedure were confirmed. The risks and benefits of surgery were discussed and I marked the left wrist with my initials. The patient was brought to the operative suite where she was administered general anesthesia. A non-sterile tourniquet was placed on the left upper extremity. The left upper extremity was prepped and draped in a standard sterile fashion. A brief time-out was performed to verify the correct patient, laterality, and procedure. All were in agreement. An Esmarch was used and the tourniquet was inflated to 230 mmHg. I started with the volar mass. I made a longitudinal incision along the FCR and then made another incision along the plane of the thenar muscle centered over the mass. I incised the dermis. I spread and identified several small arterial branches which were cauterized and mobilized. I identified the mass. I traced it down to the radiocarpal joint capsule. I cauterized it at the stalk and sent it as a specimen to Pathology. I then explored and found no residual mass. I irrigated the area with curette and used a 3-0 Vicryl suture to sew up the hole. This completed my left volar mass excision.

Then, I turned my attention to the dorsal wrist/hand. There was a large swelling and protuberance over the dorsal hand. I made a large longitudinal incision over the hand. I incised the dermis. I spread down. There was large protuberant mass which was encompassing all the extensor tendons of the fourth compartment. I isolated the tendons initially to the index, middle, ring, and small. The ring and small finger tendons were completely ruptured in an attritional manner with small fibrous bands bridging the tendon. The mass was very inflamed, protuberant tenosynovitis with gross rice bodies. As much of this was debulked and removed as possible, I completed the tenosynovectomy of the index, middle, ring, and small fingers. This was sent as a composite specimen to Pathology. The index and middle finger tendons were frayed but were intact. The ring and small fingers were not. I resected the bridging synovial bands. I trimmed and freshened up the edges of the ring and small finger tendons. I then used 4-0 MaxBraid suture and did a four-portion repair to repair the remaining tendons back to one another. The remainder of the bulkiness was inflamed synovium. In order to remove this, I would have to remove the capsule and I did not feel safe or comfortable doing this. At this point, I determined I had to remove as much of the mass in the form of tenosynovitis as I could safely do with this approach. The tourniquet was down.

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OPERATIVE REPORT

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Hemostasis was achieved using bipolar electrocautery. Copious amounts of sterile saline were used to irrigate the wound. Then, 3-0 Vicryl was used for dermal layer followed by 4-0 nylon horizontal mattress suture on both sides. Then, 0.25% Marcaine with epinephrine was administered for peri-incisional anesthesia. A sterile dressing was applied followed by resting hand splint in a protected position. The patient tolerated the procedure well and sustained no intraoperative complications. The patient was taken to PACU in a stable condition.

X_____
Kevin Golshani, M.D.

JOB#: 180445 KG: med: sas/rs DD: 04/12/2024 DT: 04/13/2024